

[ADDRESS]

C O N F I D E N T I A L

Daycase Discharge Summary

Division B

Care Group - Emergency & Specialist Medicine Respiratory Medicine

[PROVIDER-NAME]

MR MALCOLM MAIR

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Address

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Sex

Male

Date Of Birth

[DATE] (60 years)

Hospital No

[MED-ID]

NHS No

[NHS-NO]

Telephone Number

Admit Date and Time

[DATE] 13:45

Discharge Date

[DATE]

Admit Method and Source

Elective booked - Usual Residence

Discharge Method

On Clinical Advice Or With Consent

Hospital Site

[ADDRESS]

[ADDRESS]

Usual Place Of Residence

Consultant and Specialty

DR B MARSHALL (Respiratory Medicine)

Discharge Address

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

1. Sarcoidosis

Death Details

Action

Disease or condition directly leading to death

Other disease or condition leading (Ia)

Other disease or condition leading (Ib)

Other significant conditions CONTRIBUTING TO THE DEATH but not related to the disease or illness

Procedures

[DATE]

Patient specifically admitted for education for self-management of respiratory health - Nurse Clinic

Co-Morbidities

1. Crohns Disease

GP and Community Team - Actions and Information

None

Memory Assessment

Advance Directives And Anticipatory Care Plans

Health Promotion

1. Smoking History - Does not smoke

2. Weight: 103 kg, Height: 184 cm, BMI: 30.4

Safety Alerts

None

Social Context

1. Does Not Live Alone

Most Recent Results to Inform GP of Continued Management

Specialist Management - Acute Kidney Injury

Highest AKI stage recorded during admission

Medicines Reviewed

Medicines Review

Additional Notes

Follow Up U+Es including Creatinine and eGFR

Advisory Notes

Correct at the time of signing

Key Events During Admission

None

Investigations During Admission

None

Management Plan

Recent cold with yellow sputum. Not treated with antibiotics. Very physical job working 11 hrs driver. Feels fatigued. Weight gain 3.8kgs since last June. S/B Gastroenterologist [PROVIDER-NAME] 12th. Discussed with [PROVIDER-NAME]- bloods today. Continue on 50mgs Azathioprine od until next ILD OP July 7th.

Outstanding Investigations

You will be advised of the results when they are available

No drugs on discharge

Allergies and Adverse Reactions

None Stated

Changes To Medication

None

Weight

Medication (Approved Name)

Drug Name

Dose, Form, Frequency & Route

Days Given

GP to Continue

Pharmacy Only

*One stop dispensing indicates supply made during hospital stay Pharmacy supply between 14 and 18 days of a patient's regular medication on discharge

If you would like any further advice on your medicines you can phone our Medicines Helpline [PROVIDER-NAME] or talk to your community pharmacist (chemist)

[ADDRESS]

Controlled Medication (Approved Name)

Drug Name

Dose And Frequency

Days Given

GP to Continue
Pharmacy Only
Other Drug Information
Pharmacy Name
Bleep
Signed
Pharmacy [DICTATION-META]
Checked By
Dispensed By
Nurse Assessment
Mental Capacity: (please give details)
Dietary Requirements:
Swallow/Nutrition:
Liquids:
Solids:
Malnutrition Universal Screening Tool Score:
Pressure Ulcer Status:
Washing and Dressing:
Domestic Activities:
Continence:
Night Care:
Other Information:
Therapist Assessment
Patient's Living Arrangements:
Mobility at Discharge:
Cognitive Abilities:
Assessment of Daily Living Skills on Admission (BARTHEL):
Assessment of Daily Living Skill on Discharge (BARTHEL):
Equipment Needs:
Type of Care Package Arranged:
Please state follow up services required: e.g. Occupational Therapy / Physiotherapy / Communi
Team / Social Services / Community Nurse / Speech and Language / other:
Patient's Concerns, Expectations, Wishes:
Carer's Views:
Information Given to Patient and/or Authorised Representative:
Clinician Information
Keyworker
Primary Nurse
Physiotherapist
Speech and Language Therapist
Dietitian
Occupational Therapist
Clinical Psychologist
Team Contact Numbers
Consultant
Nursing Staff & Ward [PROVIDER-NAME]
SALT & Dietitian
Y
Information For Patient and Self Care Instructions
Preventing blood clots after discharge
The risk of developing blood clots can persist for some weeks after discharge. There are cert
look out for that could mean that you have a blood clot. You should contact your doctor immed
you develop any of the following symptoms while you are in hospital or after you go home.
Leg pain
Leg swelling
Warm or discoloured skin on the legs
Enlarged or more noticeable veins near the surface of the skin on your legs

New onset shortness of breath

Unexplained chest pain

Coughing up blood

Further Information

Please phone your GP, NHS 111 service or in the event of Emergency Telephone 999

Fit Note Duration

Follow-up

Clinic

[ADDRESS]

[ADDRESS]

To be booked by

Additional Information

Discharge Consultant

DR B MARSHALL (Respiratory Medicine)

Discharge Date

[DATE]

Name

Mrs [NAME] (Sister - Charge Nurse)

Bleep

Signed

Date

[DATE] 14:48

Copies To: